Greenfield Diagnostic Centre

Cardiology Department · NABL-accredited Laboratory · Reg. No. GDC/CARD/4471

Patient Name:	Mr. Rahul Verma	**Report No.:**	GDC-2026-008812
Age / Sex:	58 Years / Male	**Collected On:**	12 June 2026, 08:15
Patient ID:	GDC-PT-33920	**Reported On:**	12 June 2026, 17:40
Referring Doctor:	Dr. A. Nair, MD (Cardiology)	**Sample:**	Serum / Whole blood

Clinical History

58-year-old male, known hypertensive for 9 years, presenting with exertional chest tightness and breathlessness (NYHA Class II) for the past 3 weeks. Family history of coronary artery disease. Evaluated for ischemic heart disease.

Lipid Profile & Cardiac Markers

Test	Result	Unit	Reference Range	Flag
Total Cholesterol	268	mg/dL	< 200	HIGH
LDL Cholesterol	182	mg/dL	< 100	HIGH
HDL Cholesterol	34	mg/dL	> 40	LOW
Triglycerides	256	mg/dL	< 150	HIGH
Troponin I	0.06	ng/mL	< 0.04	HIGH
NT-proBNP	612	pg/mL	< 125	HIGH
hs-CRP	4.8	mg/L	< 3.0	HIGH
Fasting Blood Glucose	138	mg/dL	70 - 100	HIGH
HbA1c	7.1	%	< 5.7	HIGH
Serum Creatinine	1.0	mg/dL	0.7 - 1.3	NORMAL

ECG & Echocardiography

ECG: Sinus rhythm, rate 88/min. ST-segment depression of 1.5 mm in leads V4-V6 suggestive of lateral wall ischemia.
2D Echo: Mild left ventricular hypertrophy. Regional wall motion abnormality in the lateral wall. Left ventricular ejection fraction (LVEF) estimated at 48% (mildly reduced). Grade I diastolic dysfunction.

Impression

Findings are consistent with **coronary artery disease with myocardial ischemia** on a background of dyslipidemia, type 2 diabetes mellitus, and hypertension. Mildly elevated Troponin I and NT-proBNP indicate early myocardial strain. Coronary angiography is advised to assess the need for revascularisation (angioplasty / CABG).

Recommendations

1. Cardiology consultation for coronary angiography.
2. Initiate / optimise statin and antiplatelet therapy.
3. Strict glycaemic and blood-pressure control.
4. Lifestyle modification: low-fat diet, supervised exercise, smoking cessation.

This is a computer-generated sample report created for software testing only. Not a real patient and not for clinical use. — Dr. A. Nair, MD (Cardiology)